



Occupational Therapy: Mental health Initial Evaluation Guideline

1. Background

The Occupational Therapist (OT) is a core member of the inpatient ward multidisciplinary teams. In line with the recovery model, Occupational Therapy is based on the philosophy and evidence that individuals' can and do recover to lead meaningful, satisfying, and productive lives.

Occupational Therapy uses a holistic approach with an emphasis on function; participation and partnership to help support individuals with a mental illness develop skills, engage in activities of interest, and reach recovery goals.

The purpose of the Occupational Therapy service in inpatient mental health is to develop and implement an OT intervention plan based on thorough individual assessment to facilitate treatment, support, and discharge planning (TSDP). This includes functional, cognitive, perceptual, social, physical, and environmental assessment and intervention. The OT can also facilitate safe discharge from hospital by providing aids and equipment based on need and home assessments pre and/or post discharge.

The initial evaluation allows OTs to gather information using a standardised process. This information is used to identify goals and further OT assessment/s and intervention. The OT will formulate an action plan with the consumer which can include 1:1 support and or group intervention working towards facilitating safe discharge planning. The OT may identify the need for other professionals' input and will refer to other members of the MDT.

2. Purpose

To ensure consistent Occupational Therapy Initial evaluation, assessment, intervention, and discharge planning processes for consumers admitted to all mental health wards at Armadale Health Service.

3. Applicability

This guideline applies to occupational therapy within the inpatient mental health units at Armadale Health Service

4. Guidelines

- Initial Evaluation Form needs to be completed within seventy-two hours (three working days) from point of referral, unless unable due to clinical status / lack of ability to contact significant others.
- If the patient is re-admitted, or is being admitted at regular intervals, the Initial Evaluation Form (PAKMR61.10) can be referred to in the integrated notes if completed within past twelve months. If significant information has changed a new form will need to be completed.
- Document in the Integrated Progress Notes when an OT Mental Health Initial Evaluation has been completed and reference PAKMR61.10.

- The completed form is used to inform the TSDP and the recommendations will be entered into the inpatient management plan on WebPSOLIS.
- Contains information from:
 - Medical records including webPSOLIS, ICM re: previous OT input from medical admission. Mental Health Medical file which includes all information on patient presentation since admission.
 - Initial interview with patients
 - Information from family, carer and/or next of kin
 - Formal supports such as NDIS coordinators and supports (with permission)
 - Residential / group home supports.
- Information type:
 - Qualitative – e.g., Information collected from patient/relatives
 - Objective – e.g., Observation (formal/informal assessments)

4.1 Key to initial assessment

4.1.1 Admission

Consent:	Tick if consent is gained
Referred From:	Who referred the patient to OT e.g. member of the MDT, outcome of bariatric risk assessment, FRAMP, and Braden scale or as identified by the treating Occupational Therapist.
Referral Reason:	The reason why they were referred to OT e.g. review of occupational engagement (self-care, leisure and productivity), occupational goals, review level of function, review home environment, pressure care, falls risks, cognition, specific functional difficulty etc.
Medical History:	The portion of the patient's life history, including ancestry, social, occupational, and medical, that is important in diagnosing and caring for the medical or surgical condition/s present.

4.2 Person components

4.2.1 Cognition

- Consider Delirium and complete delirium screening using the Single Question in Delirium (SQuID) tool.
 - Do you feel that [patient's name] has been more confused lately? Ask patient or family has there been an acute change in cognitive function? Ask the patient questions related to orientation time, place, and person e.g. Do you know what day, month, and year it is? Do you know where you are? What is your name, DOB, and address? To assess attention, ask can you tell me the months of the year backwards (MOYB)? If unable to complete MOYB, ask the person to count backwards from 20-1. Complete a TOP 5 with loved ones if known diagnosis of dementia.
- Other questions that are useful to ask

- How long have you been in hospital?”, and “Why are you in hospital?”
- Can the patient give you accurate information regarding their preadmission function, and home set up, etc.
- Consider if any cognitive assessment has been completed in the past twelve months, document the assessment that was completed, total score and date of completion, in the comments section.

The Occupational Therapist will use their clinical reasoning to determine if further specific cognitive / executive or functional assessments are appropriate during the inpatient admission. Assessment including MOCA, HDS-R, RUDAS, Multiple errands, ACERIII, functional kitchen assessment and medication skills assessment may be deemed appropriate depending on the discharge goals. The OT will document if further assessment is required in the Actions list at end of the Initial Evaluation Form.

4.2.2 Functional mobility

Functional mobility refers to the ability of a person to move from a starting point into a pre-determined position, this includes:

Bed Mobility:	Rolling, bridging, ability to move up the bed, repositioning in bed.
Transfers/Seating:	Sit to stand, on/off chair /bed/ toilet, in / out of car, reposition while seated.
Ambulation:	Including equipment required, wheelchair mobility, safety when walking.
Falls History:	Ask about the number of falls in the past 12 months. Ask details of falls – trip, legs gave way, loss of consciousness, location, injuries, etc.
Equipment:	Required for self-care, ambulation, seating, pressure care, etc.

All patients who weigh ≥ 120 kg, or with a body mass index (BMI) > 35 will have a bariatric patient initial risk assessment completed (AKMR 111.2.1). Document this in the Actions list at the end of the Initial Evaluation form.

4.2.3 Sensory

Vision:	Ask patient about their vision and if any aids are required. Note bifocals can increase falls risk.
Hearing:	Ask patient about their hearing and if any aids are required. Do they have any difficulty managing the aids?
Pressure Injury Prevention & Management:	Review Skin integrity Assessment and Braden score. Identify pressure injury risk and educate on risks. If clinically indicated prescribe equipment provisions i.e: support surface, air mattress, heel elevator, chair cushion etc.

Depending on needs for sensory modulation intervention; patient may require a Sensory Profile assessment. Document this in Actions list at end of the Initial Evaluation Form.

4.2.4 Occupation / roles

Occupational roles refer to a person's primary responsibilities for example: worker, student, carer, or spouse. Occupational roles contribute to personal identity and use of time.

- Ask the patient: what do you do? what are your main responsibilities?
- For each role, discuss importance, how well they can fill the role and enjoyment?
- Does the patient fulfil multiple roles?
- Determine if the patient is able to fulfil a range of role responsibilities or any barriers to participation.

Depending on patient's ability to identify roles; patient may require role checklist. Document this in Actions list at end of the Initial Evaluation Form.

4.2.5 Leisure/interests

Leisure refers to participation in activities outside of main occupational role (student, worker, carer etc).

- Ask the patient aside from your main occupational role, what do you like to do with your time? Do you have any interests or hobbies? How often do you participate in these activities and are you satisfied with the amount of time?
- If patient cannot express what they enjoy, ask them what have they done in the past that they enjoyed?
- If the patient can express interest but are not participating, ask them what is preventing them from doing so (e.g. money, transportation, safety concerns or physical barriers).

Depending on needs for leisure development; patient may benefit from completing an Interest Checklist. Document this in Actions list at the end of the Initial Evaluation form.

4.2.6 Psychiatric symptoms

Positive psychiatric symptoms are described as those that are 'added' to the individual's experience of life; feelings or behaviours that are not actually present e.g. hallucinations (seeing, hearing, tasting, smelling or feeling something that is not there) and delusions (odd or unusual beliefs usually seen as untrue by others).

Negative psychiatric symptoms can be described as a lack of feelings or behaviours e.g. a lack of pleasure in activities previously enjoyed, reduced speaking, lack of emotion, incongruent affect, loss of interest in socialising or relationships and self-neglect e.g. not showering, preparing meals or cleaning.

Positive/Negative Symptoms:	Ask the patient if they have experienced anything unusual or what led to their admission. If a patient does not have insight this information can be gathered from the NOK or carer and/or observations as an inpatient.
Impact on Performance:	Ask the patient how the symptoms they experience impact on their daily tasks and participation in meaningful activities. Include observations and reports from carer/NOK.

Coping strategies: Ask the patient what coping strategies they use currently or have used in the past to help them in managing their symptoms. Ask them what strategies they have tried but haven't worked, and why.

4.2.7 Routine

Routine refers to a sequence of actions, tasks and/or activities that are regularly followed. Routine characterises an individual's daily patterns and provides a degree of structure and predictability to life.

Balance: Between work, leisure, social, personal, physical and creative. Ask the patient what they would like their routine to be like and are they satisfied with their current routine?

Structure: Daily and/or weekly schedule, regular activities. Ask the patient to describe a typical day or week, weekday vs. weekend, what is their current routine? Sleep patterns. Ask the patient 'How is your sleep pattern now?' What time does the patient usually sleep and wake?

Productivity: Amount of activity, use of time, ability to keep appointments, etc. Has the patient's routine changed and how? Is the patient able to arrange an organised and productive routine of daily activities? Is the patient meeting role responsibilities and/or goals?

4.2.8 Personal causation

Personal causation refers to a) an individual's sense of competence or personal capacity and, b) their potential effectiveness in using physical, intellectual and social capabilities to achieve desired outcomes (Kielhofner, 2008, p. 35; Parkinson, Forsyth, & Kielhofner, 2015, p. 4).

Self-Appraisal of Ability: Ask the patient what they believe their strengths and limitations are; physical, intellectual, social. 'What is something you have done well or are proud of?' Does the patient over/underestimate their abilities? Does the patient have confidence in their abilities?

Expectation of Success: Ask the patient 'How successful do you think you will be over the next six months?' What challenges are they currently facing; what is their expectation of outcomes? Does the patient anticipate success and seek challenges? Are they optimistic about overcoming obstacles? Does the patient accept circumstances beyond their control without discouragement? Is there a strong belief in personal effectiveness?

Interest: Does the patient show enjoyment, satisfaction, and curiosity? Is there passive or active interest in available occupations? Has the patient expressed or demonstrated pleasure in occupations? N.B Participation in meaningful activity is assessed in Leisure/Interests.

Choices: Does the patient have clear preferences and sense of what is important? Do priorities and personal standards match societal values? Is there appropriate commitment, initiative and persistence to work towards goals? What is the patient's ability to prioritise and take

appropriate action? Has the patient demonstrated spontaneity and reactivity to the environment?

Substance use: Are they using alcohol or any other illicit or legal drugs (including nicotine, caffeine, prescription, over-the-counter drugs, and nicotine) and what is their motivation for doing so? Do they have insight into how their substance use is affecting their mental health? Are they motivated, at present, to change their substance use? If so, what strategies and/or supports do they have to help them do this.

4.2.9 Communication/interaction

Communication and interaction refer to the ability to understand and convey intentions and needs to/from others. It can be verbal or non-verbal including gesturing, physical contact, speaking, engaging, and collaborating with others, and asserting oneself (Kielhofner, 2008).

This area can be assessed through interview and observation of the patient in groups, while on the inpatient unit and in the community setting if appropriate.

Verbal & Non-Vocal Skills: Can they express their thoughts and needs? Are they able to convey their mood and make needs known? Monitor tone and volume of speech. Is English their first language? Eye contact, body language, gestures, facial expressions, awareness of personal space, etc.

Conversation: Can they understand what is being conveyed? Are they reactive and spontaneous? Are they able to maintain discussion?

Literacy: Can they read, write, or sign their name?

4.2.10 Social environment

Social environment refers to the people and/or groups in the person's environment that do/don't support engagement in activities of daily living, work, and leisure.

Social history/ Current Social Situation: Ask the patient about their access to and engagement with family, groups or contacts including interactions with groups or communities (e.g. cultural, spiritual or activity based). Is your social setting supportive, do the people in your life place limits on you?

Relationships: Does the patient prefer to work alone or with others and how well? Do they feel comfortable in a group situation? Are they able to develop and maintain relationships with friends, family, colleagues, co-patients, staff etc.?

Current Supports/Services: Ask the patient if they are engaging with any services or supports who assist and facilitate occupational participation. Ask the patient, who are the important people in your life? Do you have the resources and opportunities to do the things you want to do?

Depending on needs for social support; patient may require referral to individual support programs within the community. Document this in Actions list at end of the report.

4.3 Home environment

Home environment refers to the physical layout and structure of the location in which the patient currently resides and/or will reside in post discharge.

Type:	Owned or rental (private/ homes west), single or double story, park home, granny flat, group home, etc.
Access:	Number steps front and back entry, internal steps, ramps, rails
Bathroom:	Shower recess or bath/shower, over bath/roman bath, shower hob, existing rails and equipment
Toilet:	Access, existing rails and equipment, distance from bed
Bedroom:	Type of bed, existing rails and equipment in situ (including commode, urinal bottle, transfer aids).
Seating:	Type of chair, height, armrests, equipment
Other:	Use to describe safety issues or hazards in the home (loose rugs, no bedside lamp etc.), describe other areas of home such as kitchen or laundry

Depending on identified environmental barriers (hazards, need for equipment); patient may require a Home Assessment and/or Equipment to be provided on discharge. Document this by ticking the boxes in the Home Environment section and document in Actions list at end of report.

4.4 Occupation/performance

The ability to perceive, desire, recall, plan and carry out roles, routines, tasks, and sub-tasks for the purpose of self-maintenance, productivity, leisure, and rest in response to demands of the internal and/or external environment.

- Rate the patients baseline and current function in each area according to the 'Key' and comment in the space provided e.g. equipment required, assistance provided and by whom.
- If a Modified Barthel Index (MBI – AKMR 61.2.1) or Functional Independence Measure (FIM) AKMR 61.2.2 has been completed or scores are available from previous admissions, enter the score here.
- Under current function describe any differences between baseline function and functioning on the ward e.g. 'Dressing upper body taking longer than usual due to shortness of breath', 'Sitting to shower on ward, with minimal assistance from nursing staff to wash back', or 'Verbal prompting/encouragement required to shower due to low mood and lack of motivation'.

4.4.1 Personal / Self-Care (ADLs)

Self-care refers to an individual's ability to maintain independence in matters concerning bodily functions and grooming.

Showering:	Equipment / aids used; assistance usually provided
Toileting:	Continence, aids used, night toileting – frequency / urgency
Un/Dressing:	Upper and lower body, aids used, assistance provided
Grooming:	Brushing hair and teeth, makeup, shaving
Eating:	Ability to use a knife, fork and spoon, adaptive equipment used
Medication:	Webster pack, dosette box, dosage and routine, appointments with doctor and other mental health professionals, filling prescriptions at the pharmacy. Consider the need for medication assessment and use of tools to develop medication routines.
Sleep:	Pattern, routine, duration, disruptions, quality

4.4.2 Instrumental / Domestic (IADLs)

Domestic tasks refer to the management of duties involved in running a household; it is a composite of physical tasks and managerial functions.

Shopping/Errands:	Level of independence, assistance provided and by whom.
Meal Preparation:	Type of meals prepared (from scratch, frozen dinners etc.). Simple vs complex meals, assistance provided and by whom.
House Cleaning/gardening:	Level of independence, assistance provided and by whom.
Laundry:	Level of independence, assistance provided and by whom.
Pet Care:	Type and number. Who is currently looking after the pets? Level of independence, assistance provided and by whom.
Financial Tasks:	Bill paying etc. Level of independence or assistance provided by guardian / public trustee / caregiver.
Transport:	Licence, vehicle, public transport, community access, car transfer.

4.5 Patient goals

- Goals can be short or long term. Discuss how the goals can be accomplished; are they realistic and achievable?
- Collaborate with and encourage patients to set treatment and recovery goals that are meaningful to them; this has been linked to increased sense of autonomy, improved motivation, and participation in a rehabilitation and/or recovery program.
- Ask the patient what they hope to achieve in the next week, month, or year.
- What does the patient want their future to look like? What do they hope to achieve prior to discharge?

4.6 Recommendations/plan

Identify areas of concern related to occupational therapy and list the actions to be completed e.g. further assessment, individual intervention, information or education, groups, referral to other professions, liaison with team and family, equipment provision, etc. Enter recommendations and plan into the inpatient management plan to inform the multidisciplinary team and treatment, support, and discharge planning.

5. Roles and responsibilities

Role	Responsibility
Senior OTs	Education to all new OT staff members during orientation on policies. Regular updates in OT staff meetings. Updating policy as required to maintain currency and accuracy.
All OT Department Staff Members	To have read this policy as per the orientation checklist for their clinical area.

6. Legislative context

The following documents give effect to this guideline:

- [EMHS Comprehensive Care Policy](#)
- [EMHS Policy Guidelines](#)

7. Supporting information

The following documents support the implementation of this policy:

- [Occupational Therapy Mental Health Procedure](#)
- [Occupational Therapy Home Assessments Procedure](#)
- [Occupational Therapy Pressure Injury Prevention and Management Procedure](#)
- [Bariatric Management Procedure](#)
- [Occupational Therapy Home Installations, Fabrications and Modifications Guideline](#)
- [Allied Health Provision of Assistive Technology and Home Modifications Procedure](#)
- [East Metropolitan Health Service Patient Identification and Procedure Matching Policy](#)
- [Montreal Cognitive Assessment \(MOCA\) Guideline](#)
- [Fall and Fall Injury Prevention and Management Guideline](#)
- [Delirium Clinical Management Guideline](#)

8. Relevant standards

National Safety and Quality Health Service (NSQHS) Standards (second edition):

Standard 1 – Clinical Governance Standard

Governance, leadership, and culture

- Action 1.01: The governing body: (a) Provides leadership to develop a culture of safety and quality improvement and satisfies itself that this culture exists within the organisation. (b) Provides leadership to ensure partnering with patients, carers, and consumers. (c) Sets priorities and strategic directions for safe and high-quality clinical care and ensures that these are communicated effectively to the workforce and the community. (d) Endorses the organisation's clinical governance framework. (e) Ensures that roles and responsibilities are clearly defined for the governing body, management, clinicians, and the workforce. (f) Monitors the action taken as a result of analyses of clinical incidents. (g) Reviews, reports and monitors the organisation's progress on safety and quality performance

Standard 2 – Partnering with Consumers Standard

Clinical governance and quality improvement systems to support partnering with consumers

- Action 2.01: Integrating clinical governance: Clinicians use the safety and quality systems from the Clinical Governance Standard when: a. Implementing policies and procedures for partnering with consumers b. Managing risks associated with partnering with consumers c. Identifying training requirements for partnering with consumers

Standard 5 – Comprehensive Care Standard

Preventing and managing pressure injuries

- Action 5.21: The health service organisation providing services to patients at risk of pressure injuries has systems for pressure injury prevention and wound management that are consistent with best-practice guidelines

Preventing falls and harm from falls

- Action 5.24: The health service organisation providing services to patients at risk of falls has systems that are consistent with best-practice guidelines for: (a) Falls prevention. (b) Minimising harm from falls. (c) Post-fall management

Minimising restrictive practices: restraint

- Action 5.35: Where restraint is clinically necessary to prevent harm, the health service organisation has systems that: (a) Minimise and, where possible, eliminate the use of restraint. (b) Govern the use of restraint in accordance with legislation. (c) Report use of restraint to the governing body

National Standards for Mental Health Services (2010):

Standard 3 – Consumer and carer participation

Consumers and carer are actively involved in the development, planning, delivery and evaluation of services

- Criteria 3.1: The MHS has processes to actively involve consumers and carers in planning, service delivery, evaluation and quality programs.
- Criteria 3.2: The MHS upholds the right of the consumer and their carer(s) to have their needs and feedback taken into account in the planning, delivery and evaluation of services.

Standard 6 – Consumers

Consumers have the right to comprehensive and integrated mental health care that meets their individual needs and achieves the best possible outcome in terms of their recovery

- Criteria 6.5: Consumers have the right to receive the least restrictive treatment appropriate, considering the consumer's preference, the demands on carers, and the availability of support and safety of those involved.
- Criteria 6.7: Consumers are partners in the management of all aspects of their treatment, care and recovery planning.
- Criteria 6.8: Informed consent is actively sought from consumers prior to any service or intervention provided or any changes in care delivery are planned, where it is established that the consumer has capacity to give informed consent.
- Criteria 6.10: Consumers have the right to choose from the available range of treatment and support programs appropriate to their needs.
- Criteria 6.11: The right of consumers to involve or not to involve carers and others is recognised and respected by the MHS.

Standard 10 – Delivery of care

Assessment and Review: Consumers receive a comprehensive, timely and accurate assessment and a regular review of progress is provided to the consumer and their carer(s)

- Criteria 10.4.2: Assessments are conducted during the consumer's first contact with the MHS by appropriately qualified staff experienced and trained in assessing mental health problems, and where possible in a consumer's preferred setting with consideration of safety for all involved.
- Criteria 10.4.4: The MHS actively plans as early as possible in the course of psychiatric inpatient admission, for the discharge of the consumer from inpatient care.

9. References

1. Australian Government. (2010). National Standards for Mental Health Services. Barton, ACT: Australian Government. Retrieved from National Standards for Mental Health Services 2010 and implementation guidelines | Australian Government Department of Health
2. Forsyth, K., Deshpande, S., Kielhofner, G., Henriksson, C., Haglund, L., Olsen, L., ... Kulkarni, S. (2006). A user's manual for the occupational circumstances assessment interview and rating scale (OCAIRS). Chicago, IL: MOHO Clearinghouse.
3. Kielhofner, G. (2008). Model of human occupation: Theory and application (4th ed.) Baltimore, MD., Philadelphia, PA: Lippincott Williams & Watkins.
4. Parkinson, S., Forsyth, K., & Kielhofner, G. (2004). A user's manual for model of human occupation screening tool (MOHOST). Chicago, IL: MOHO Clearinghouse.

10. Document contact

Enquiries relating to this guideline may be directed to:

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11. Review

This guideline will be reviewed and evaluated as required to ensure relevance and currency. At a minimum it will be reviewed within one (1) year after first issue and at least every three (3) years thereafter.

Version	Effective from	Effective to	Amendment(s)
V2	01/11/2023	01/11/2026	Scheduled review – minor amendment

12. Authorisation

This guideline has been authorised and issued by:

Authorised by	Karen Elliott – Service Director Mental Health
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